

CLINOMA PLATFORM

MATCHING ASSESSMENT

Day 3 Camp Quiz Booklet

Interactive Diagnosis & Classification

Prepared by: Clinoma Platform Team

Set 1: Causative Organisms & Etiology (4 Points)

Column I (Questions)	[Match]	Column II (Answers)
1. Infectious Mononucleosis	[]	A. Causative Organism: Human herpes virus-6 (HHV-6) mainly
2. Chickenpox (Varicella)	[]	B. Causative Organism: Varicella-zoster virus (VZV)
3. Roseola Infantum	[]	C. Etiology: HSV (Herpes Simplex Virus) is important cause in children.
4. Encephalitis	[]	D. Caused by Epstein-Barr virus with atypical lymphocytes

Set 2: Hallmark Clinical Presentations (4 Points)

Column I (Questions)	[Match]	Column II (Answers)
1. Measles (Rubeola)	[]	A. Characterized by high fever for 3-5 days, followed by sudden fall of temperature and appearance of maculopapular rash
2. Chickenpox (Varicella)	[]	B. Infants: bulging/tense anterior fontanel, poor feeding
3. Roseola Infantum	[]	C. Fever & itchy vesicular rash in different stages ("dew drops on a rose petal").
4. Meningitis (Acute Bacterial)	[]	D. Koplik spots (Pathognomonic, buccal mucosa, opposite 2nd molars)

Set 3: Specific Management & Complications (4 Points)

Column I (Questions)	[Match]	Column II (Answers)
1. Mumps (Epidemic Parotitis) Complications	[]	A. Vitamin A (Reduces morbidity/mortality in developing countries)
2. Measles (Rubeola) Treatment	[]	B. Orchitis, pancreatitis, meningitis, thyroiditis, mastitis, myocarditis, arthritis, hepatitis, nephritis, unilateral deafness, ocular, Mumps Embryopathy
3. Meningitis Specific Antibiotic Treatment (Infants < 6 months)	[]	C. ampicillin + gentamicin (IV)
4. Encephalitis Management	[]	D. Requires urgent antiviral/supportive treatment

Set 1: Pathophysiology & Etiology Integration (5 Points)

Column I (Questions)	[Match]	Column II (Answers)
1. Duchenne Muscle Dystrophy Pathophysiology	[]	A. Due to a defect in a protein called dystrophin of the skeletal muscle, cardiac muscle and the brain.
2. Cerebral Palsy Core Deficit	[]	B. X-linked recessive disease of late onset during late childhood, with a slowly progressive course and a longer time of survival.
3. Kernicterus	[]	C. Central motor deficit that is strictly non-progressive, non-familial, and non-hereditary.
4. Anterior Horn Cell Disease	[]	D. Werdnig-Hoffman spinal muscular atrophy.
5. Becker Muscle Dystrophy Comparison Note	[]	E. Bilirubin-induced encephalopathy.

Set 2: Clinical Signs, Postures & Motor Abnormalities (5 Points)

Column I (Questions)	[Match]	Column II (Answers)
1. Weakness of pelvic girdle muscles (in DMD)	[]	A. The infant slips through the examiner's hands when held under the arms, denoting hypotonia of the shoulder girdle muscles.
2. Slippage (Vertical Suspension)	[]	B. Arms and Legs but legs are more involved.
3. Diplegia	[]	C. When the baby is suspended in a prone position over the examiner's hand, they drop around it in a "U-shape", denoting hypotonia of the trunk muscles.
4. Dystonia	[]	D. Painful sustained contraction of a muscle group's → prolonged, twisting, posturing of the face, limb or trunk, which involved.
5. Curved Trunk (Ventral Suspension)	[]	E. Waddling lordotic gait (weak gluteus medius and minimus), Difficulty climbing up stairs, and Positive Gower sign.

Set 3: Diagnostics & Diagnostic Criteria (4 Points)

Column I (Questions)	[Match]	Column II (Answers)
1. Muscle biopsy (Diagnostic) for DMD	[]	A. Transient autoimmune neuromuscular blockade.
2. Lower Motor Unit Disorders Criteria (Deep Tendon Reflexes)	[]	B. Characterized by areflexia or hyporeflexia.
3. Spastic CP (70 to 80%)	[]	C. Shows muscle fiber degeneration with replacement with fat and fibrosis.
4. Neonatal Myasthenia Gravis	[]	D. Hypertonia with hyperreflexia, positive Babinski sign (extensor plantar response), and persistent primitive reflexes.

Set 1: Classic Murmurs & Auscultatory Findings (5 Points)

Column I (Questions)	[Match]	Column II (Answers)
1. Crucial Note in ASD	[]	A. A systolic ejection murmur at the left sternal border caused strictly by the pulmonary stenosis, not the VSD.
2. PDA Auscultation (Murmur)	[]	B. Characterized by a pathognomonic grade 1 to 4 of 6 continuous machinery murmur, best audible at the left infraclavicular area and propagated to the pulmonary area and the back.
3. ASD Heart Sounds	[]	C. There is absolutely NO MURMUR produced by the actual passage of blood through the ASD itself.
4. TOF Auscultation Murmur	[]	D. The second heart sound (S2) is widely split and fixed, which stands as the pathognomonic and characteristic finding of ASD.
5. VSD Auscultation (Murmurs)	[]	E. Harsh pansystolic murmur (lower left sternal border, propagated).

Set 2: Pathophysiology & Unique Clinical Signs (5 Points)

Column I (Questions)	[Match]	Column II (Answers)
1. The Squatting Mechanism	[]	A. Classic EGG-SHAPED ("EGG ON A STRING")
2. Large PDAs General Examination	[]	B. Pulmonary hypertension increases right ventricular pressure, causing Eisenmenger Syndrome and resulting in 'persistent cyanosis'.
3. The Essential Triad in Infants	[]	C. bounding peripheral pulses and a classic water hammer pulse, caused directly by a wide pulse pressure with elevated systolic pressure and lower diastolic pressure.
4. TGA Chest X-RAY (CXR)	[]	D. Tachypnea, Tachycardia, Enlarged tender liver
5. Eisenmenger Syndrome	[]	E. Older children squat to increase systemic vascular resistance, which reduces the right-to-left shunt and improves pulmonary blood flow.

Set 3: Protocols, Drugs & Contraindications (5 Points)

Column I (Questions)	[Match]	Column II (Answers)
1. TET SPELLS CRITICAL ABSOLUTE CONTRAINDICATIONS	[]	A. IMMEDIATE administration to keep DUCTUS ARTERIOSUS PATENT
2. Digitalis Toxicity Symptoms	[]	B. Give NO INOTROPES and NO DIURETICS during a spell.
3. ARTERIAL SWITCH OPERATION (ASO)	[]	C. Anorexia, nausea, vomiting, bradycardia, visual disturbances, severe arrhythmias.
4. PDA Medical Prophylaxis (Premature Only)	[]	D. Surgically switches Aorta and Pulmonary Artery to correct ventricles. Must occur EARLY (3-8 WEEKS).
5. INTRAVENOUS PROSTAGLANDIN	[]	E. Pharmacological closure using indomethacin, ibuprofen, or paracetamol is highly effective and widely used to close the PDA strictly in premature infants.